

CLINICAL TRIALS LANDSCAPE REPORT

Breast Cancer Clinical Trials in Australia and New Zealand

A cross-sectional analysis of currently recruiting trials using ClinicalTrials.gov API v2

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Data retrieved: 6 September 2026

Portfolio data analysis | Python, pandas, REST API, JSON and data visualisation

Executive summary

This report maps currently recruiting interventional breast cancer trials with confirmed recruiting locations in Australia or New Zealand. The analysis is a dated registry snapshot rather than a measure of treatment effectiveness or patient enrolment.

Headline result: Eighty-eight unique interventional trials had at least one confirmed recruiting site in the region. Australia participated in 87 trials and New Zealand in eight; seven trials recruited in both countries.

Australia recorded 363 recruiting study-location records compared with 14 in New Zealand. Population adjustment narrowed the difference, but did not remove it: Australia had 3.15 recruiting trials per million residents versus 1.50 in New Zealand, and 13.15 recruiting site records per million versus 2.63.

The trial portfolios also differed. Phase 3 accounted for 62.5% of New Zealand's eight recruiting trials, compared with 26.4% of Australia's 87. Australia had a broader early-phase portfolio, including 27 Phase 1/2 trials. Industry was the lead sponsor for 89.7% of Australian trials and 87.5% of New Zealand trials.

Recruitment activity was geographically concentrated. New South Wales and Victoria together accounted for 65.6% of Australian site records, while Auckland Region accounted for 57.1% of New Zealand site records. These patterns describe registered activity and should not be interpreted as direct measures of equitable access.

Objective

The objective was to identify and compare breast cancer trials that were actively recruiting at registered sites in Australia and New Zealand, then describe their geographic footprint, development phase, sponsor profile and intervention landscape.

Data source and method

Study records were retrieved from the public ClinicalTrials.gov REST API v2 on 6 September 2026. The initial query searched for the condition 'Breast Cancer' and locations matching 'Australia OR New Zealand'. The API returned JSON, which was parsed and structured with Python and pandas.

Country names were validated against each registered location rather than relying only on the text search. The primary analysis retained interventional studies with an overall status of Recruiting and at least one Australian or New Zealand location whose site-level status was Recruiting. Studies with only Not Yet Recruiting regional sites were retained separately as pipeline records but excluded from the primary current-recruitment cohort.

Table 1. Study-selection flow

Stage	Studies	Excluded
Initial API search	500	0
Confirmed AU/NZ location	499	1
Recruiting or not yet recruiting	113	386
Regional site recruiting/upcoming	97	16
Interventional only	91	6
Currently recruiting regional site	88	3

Definition note: A recruiting site record is one study-location record. It is not necessarily a unique physical hospital, because the same facility can participate in several trials and facility names may vary.

For the city-level map, coordinates were matched from ClinicalTrials.gov location records for 85 of 97 cities. Twelve remaining city labels were assigned reviewed fallback coordinates. Original registry labels were retained, while standardised map labels and location quality-control notes recorded any spelling, abbreviation or state correction.

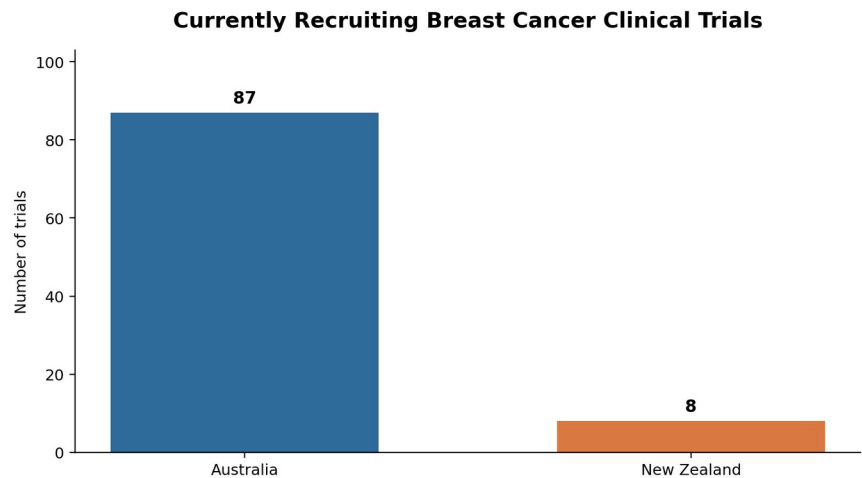
Results

1. Recruiting trial footprint

Australia participated in 87 currently recruiting trials, compared with eight in New Zealand. Seven were shared across the two countries, leaving 80 Australia-only trials and one New Zealand-only trial.

Table 2. Current recruiting footprint

Country	Trials	Site records	Facility names	Cities	Mean sites/trial	Median
Australia	87	363	216	89	4.17	3.0
New Zealand	8	14	13	7	1.75	2.0



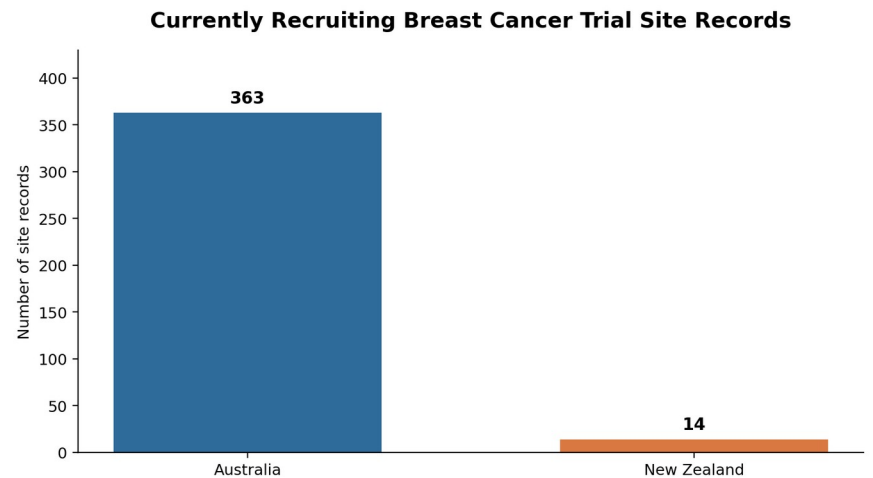
Source: ClinicalTrials.gov API v2 | Data retrieved: 6 Sep 2026

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Figure 1. Number of currently recruiting trials represented in each country.

Recruiting site records

The difference was larger at site-record level. Australia had 363 recruiting study-location records, compared with 14 in New Zealand. The mean number of recruiting sites per represented trial was 4.17 in Australia and 1.75 in New Zealand.



Source: ClinicalTrials.gov API v2 | Data retrieved: 6 Sep 2026

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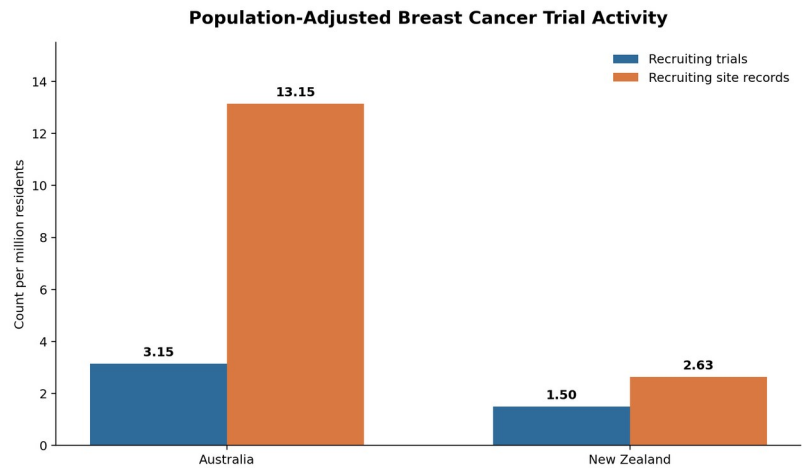
Figure 2. Recruiting study-location records by country.

Population-adjusted comparison

Raw counts partly reflect Australia's larger population. Using matched official population estimates for 30 June 2025, Australia still showed a higher registered trial footprint: 2.1 times the trial rate and 5.0 times the site-record rate per million residents.

Table 3. Population-adjusted recruitment activity

Country	Population	Trials/million	Site records/million
Australia	27,614,411	3.15	13.15
New Zealand	5,324,700	1.5	2.63



Rates describe registered trial activity, not patient enrolment or individual access.
Sources: ClinicalTrials.gov API v2; ABS and Stats NZ population estimates (30 Jun 2025)
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Figure 3. Population-adjusted recruiting trial and site activity.

2. Geographic distribution

Australian recruitment was concentrated in the two most populous participating states. New South Wales accounted for 139 records and Victoria for 99; together they represented 65.6% of all Australian recruiting site records. No currently recruiting sites were identified in the Australian Capital Territory or Northern Territory in this snapshot.



Source: ClinicalTrials.gov API v2 | Data retrieved: 6 Sep 2026

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Figure 4. Recruiting site records by Australian state.

New Zealand regions

New Zealand's 14 recruiting site records were spread across six regions, but more than half were in Auckland Region. Wellington Region had two records, while Bay of Plenty, Canterbury, Manawatu-Whanganui and Waikato each had one.



Source: ClinicalTrials.gov API v2 | Data retrieved: 6 Sep 2026

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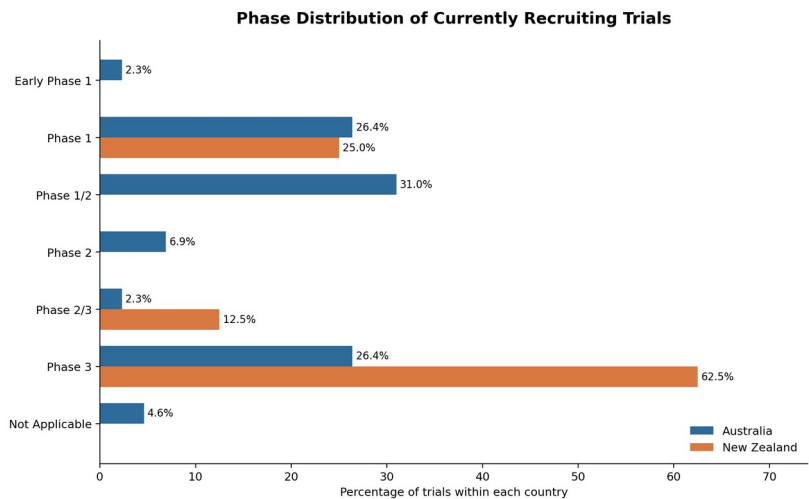
Figure 5. Recruiting site records by New Zealand region.

3. Trial development phase

Australia's portfolio spanned the full development pathway, with Phase 1/2 the largest category at 31.0%. New Zealand's smaller portfolio was weighted toward later-stage research: five of eight trials were Phase 3. Because each New Zealand trial represents 12.5 percentage points, these proportions are descriptive and should be interpreted cautiously.

Table 4. Phase distribution within each country

Phase	AU n	AU %	NZ n	NZ %
Early Phase 1	2	2.3	0	0.0
Phase 1	23	26.4	2	25.0
Phase 1/2	27	31.0	0	0.0
Phase 2	6	6.9	0	0.0
Phase 2/3	2	2.3	1	12.5
Phase 3	23	26.4	5	62.5
Not applicable	4	4.6	0	0.0



Percentages calculated within each country (Australia n=87; New Zealand n=8).
Source: ClinicalTrials.gov API v2 | Data retrieved: 6 Sep 2026

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Figure 6. Percentage distribution of development phases within each country.

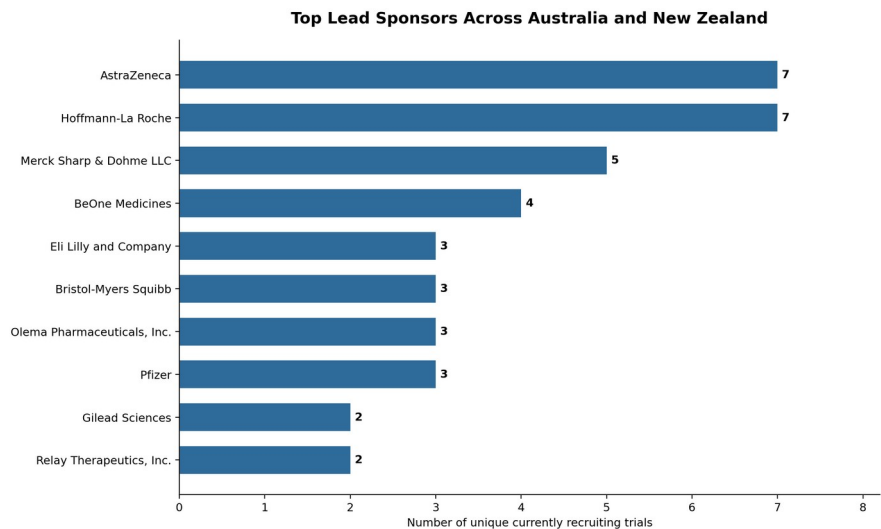
4. Sponsor profile

Lead sponsorship was overwhelmingly commercial in both countries. Industry led 78 of Australia's 87 represented trials and seven of New Zealand's eight. The similarity suggests that the large difference in trial volume is not explained by a different industry-versus-other sponsor mix.

Table 5. Lead sponsor class

Lead sponsor class	Australia n	Australia %	New Zealand n	New Zealand %
Industry	78	89.7	7	87.5
Other	9	10.3	1	12.5

Across the combined set of 88 unique trials, AstraZeneca and Hoffmann-La Roche each led seven trials, followed by Merck Sharp & Dohme with five and BeOne Medicines with four.



Combined analysis of 88 unique trials; counts refer to the registered lead sponsor only.
Source: ClinicalTrials.gov API v2 | Data retrieved: 6 Sep 2026

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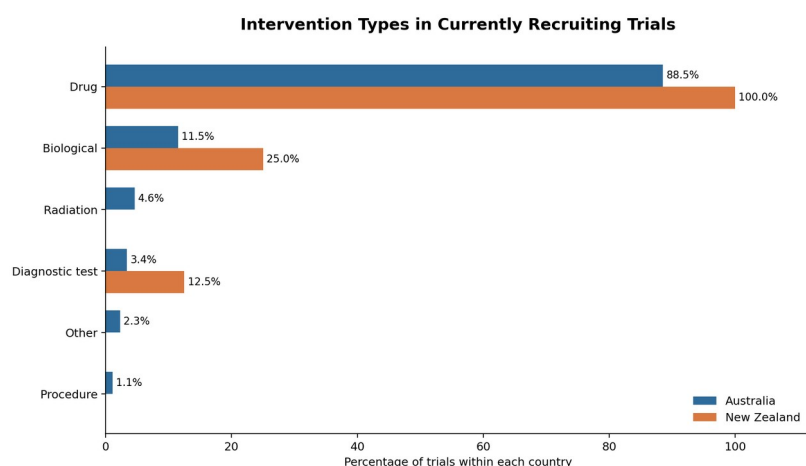
Figure 7. Leading sponsors across the combined regional trial set.

5. Intervention landscape

Drug interventions dominated the active portfolio. All eight New Zealand trials and 88.5% of Australian trials included at least one drug. Biological interventions appeared in 25.0% of New Zealand trials and 11.5% of Australian trials. A trial can include multiple intervention types, so category percentages are not mutually exclusive.

Table 6. Intervention types within each country

Type	AU n	AU %	NZ n	NZ %
Drug	77	88.5	8	100.0
Biological	10	11.5	2	25.0
Radiation	4	4.6	0	0.0
Diagnostic test	3	3.4	1	12.5
Other	2	2.3	0	0.0
Procedure	1	1.1	0	0.0



Trials may include multiple intervention types; percentages may exceed 100% (Australia n=87; New Zealand n=8).
Source: ClinicalTrials.gov API v2 | Data retrieved: 6 Sep 2026

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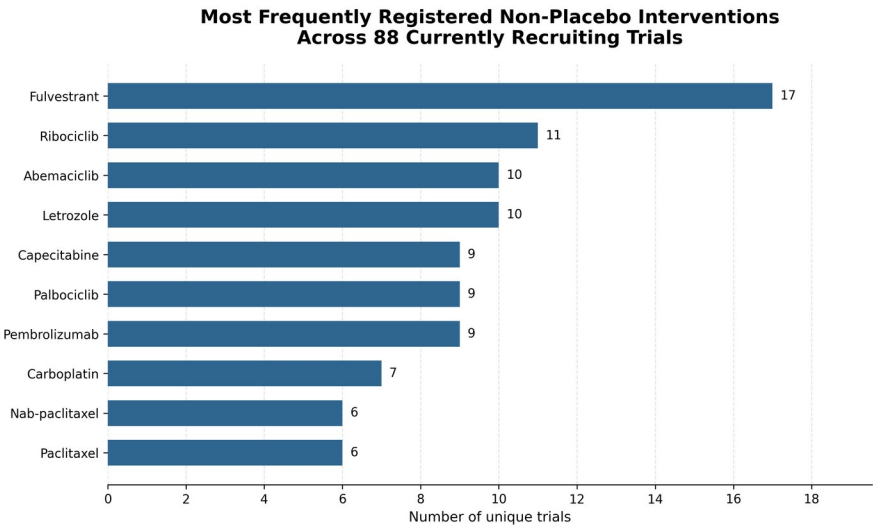
Figure 8. Intervention-type percentages within each country.

Most frequently registered interventions

After standardising differences in capitalisation, fulvestrant was the most frequently named non-placebo intervention, appearing in 17 of the 88 unique trials (19.3%). Ribociclib appeared in 11 trials (12.5%), while abemaciclib and letrozole each appeared in 10 (11.4%). Capecitabine, palbociclib and pembrolizumab each appeared in nine trials (10.2%). These counts do not distinguish experimental therapy from comparator or background therapy.

Table 7. Top non-placebo interventions across 88 unique trials

Intervention	Trials	% of 88
Fulvestrant	17	19.3
Ribociclib	11	12.5
Abemaciclib	10	11.4
Letrozole	10	11.4
Capecitabine	9	10.2
Palbociclib	9	10.2
Pembrolizumab	9	10.2
Carboplatin	7	8.0
Nab-paclitaxel	6	6.8
Paclitaxel	6	6.8



Each trial was counted once per intervention; capitalisation variants were standardised.
Independent analysis by Zohreh Riahi | Source: ClinicalTrials.gov API v2 | Data retrieved 6 September 2026

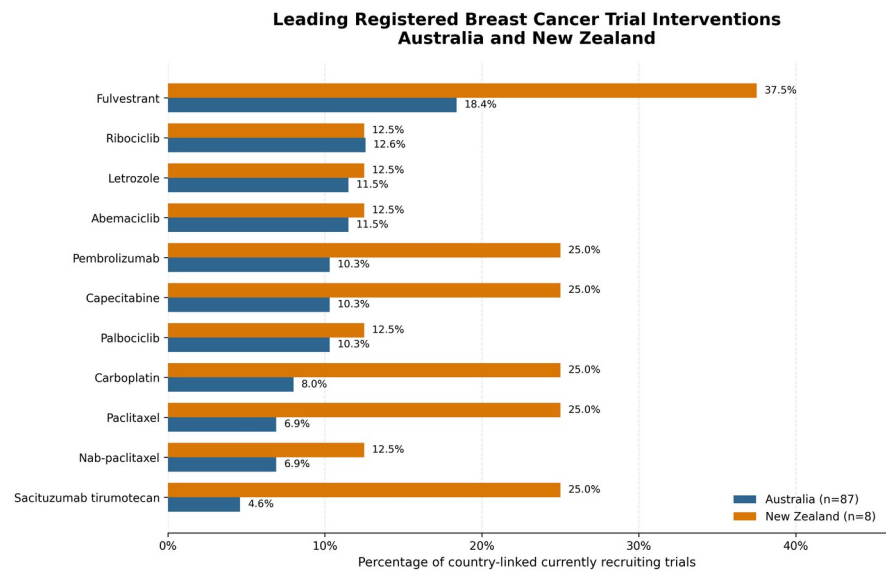
Figure 9. Most frequently named non-placebo interventions in the combined trial set.

Country-level intervention comparison

Fulvestrant appeared in 16 of 87 Australia-linked trials (18.4%) and three of eight New Zealand-linked trials (37.5%). Capecitabine, carboplatin, paclitaxel, pembrolizumab and sacituzumab tirumotecan each appeared in two New Zealand-linked trials (25.0%). Because seven of the eight New Zealand-linked trials also recruited in Australia, the country columns overlap and should not be interpreted as independent portfolios.

Table 8. Leading registered interventions within each country

Intervention	AU n	AU %	NZ n	NZ %
Fulvestrant	16	18.4	3	37.5
Ribociclib	11	12.6	1	12.5
Abemaciclib	10	11.5	1	12.5
Letrozole	10	11.5	1	12.5
Capecitabine	9	10.3	2	25.0
Pembrolizumab	9	10.3	2	25.0
Palbociclib	9	10.3	1	12.5
Carboplatin	7	8.0	2	25.0
Paclitaxel	6	6.9	2	25.0
Nab-paclitaxel	6	6.9	1	12.5
Sacituzumab tirumotecan	4	4.6	2	25.0



Independent analysis by Zohreh Riahi | Source: ClinicalTrials.gov API v2 | Data retrieved 6 September 2026

Figure 10. Percentage of country-linked trials containing each selected intervention.

Selection includes Australia's top 10 interventions plus interventions appearing in at least two New Zealand-linked trials; the generic registry label 'Rescue Medication' was excluded.

6. Interactive city-level trial map

City-level mapping identified 97 registered recruiting locations: 90 in Australia and seven in New Zealand. Melbourne had the highest city-level count with 33 unique trials, followed by Adelaide with 21, Darlinghurst and Nedlands with 17 each, and Sydney with 15. Auckland recorded six unique recruiting trials, the highest city-level count in New Zealand.

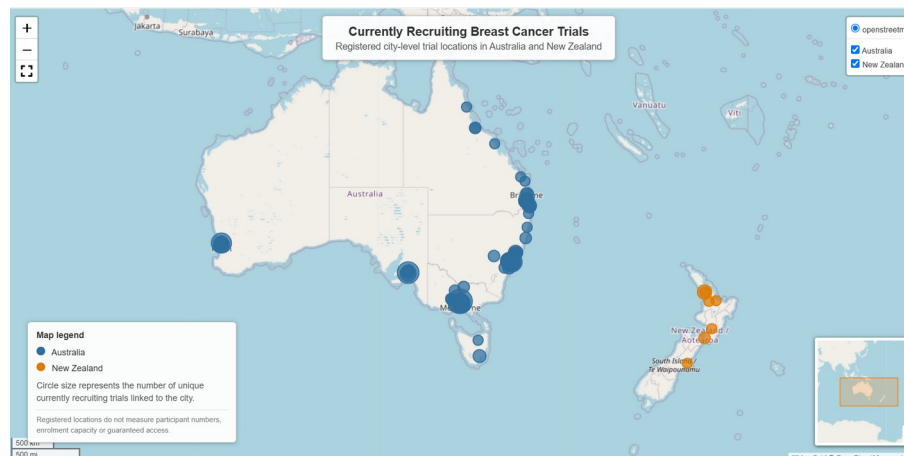


Figure 11. City-level distribution of currently recruiting breast cancer trials in Australia and New Zealand.

Marker size represents unique trials linked to each city. Individual trials may be represented in multiple cities; city-level counts should not be summed to estimate the regional trial total. Registered locations do not indicate participant numbers, enrolment capacity or guaranteed access.

Independent analysis by Zohreh Riahi | Source: ClinicalTrials.gov API v2 | Data retrieved 6 September 2026

Interactive companion map. The accompanying HTML file allows users to select a city, review registered facility labels and open the linked ClinicalTrials.gov study records.

Interpretation

The registry snapshot points to a substantial difference in both trial breadth and site density. Population adjustment reduces the raw country gap, showing why absolute counts alone are insufficient; however, Australia's per-capita trial rate remained about twice New Zealand's and its site-record rate was five times as high.

The geographic concentration suggests that registered trial opportunities may be harder to reach outside major metropolitan regions. This is especially visible in New Zealand, where eight of 14 recruiting site records were in Auckland Region. Geographic presence alone does not establish patient access, but it identifies where further access, travel-burden and equity analysis would be valuable.

New Zealand's portfolio was more heavily weighted toward Phase 3 trials, whereas Australia had substantially more early-phase activity. This may reflect differences in market size, specialist infrastructure, sponsor strategy and site capacity, but those explanations were not tested in the present dataset.

Limitations

This analysis has six important limitations. First, it is a cross-sectional snapshot and registry statuses can change after retrieval. Second, ClinicalTrials.gov is not the only registry used in Australia and New Zealand; trials registered exclusively in ANZCTR or another registry may be absent. Third, the condition query can include multi-tumour studies in which breast cancer is only one eligible cancer type. Fourth, facility naming is not perfectly standardised and site-record counts are not unique hospital counts. Fifth, population-adjusted rates are descriptive and are not adjusted for breast cancer incidence, eligible patient numbers, age structure, travel distance or actual enrolment. Sixth, city coordinates are approximate and are intended for geographic visualisation rather than clinical navigation.

Conclusion

Conclusion: Australia had a larger and more geographically extensive registered breast cancer trial footprint than New Zealand, even after population adjustment. New Zealand's smaller portfolio was concentrated in Auckland and weighted toward Phase 3 studies. The findings support closer investigation of regional access, early-phase research capacity and cross-country trial participation.

Sources

ClinicalTrials.gov Data and API: [ClinicalTrials.gov API documentation](#)

Australian Bureau of Statistics, National, state and territory population, June 2025: [ABS population release](#)

Stats NZ, National population estimates at 30 June 2025: [Stats NZ population release](#)